

Supplement Checklist

Evidence summary, cautions, quality checklist and your doctor-conversation sheet · Module 2, Training 7

The rule

Food first. Supplements fill gaps food can't reliably fill (folic acid) or gaps that testing reveals (vitamin D, iron). **What you take and how much is decided with your doctor, midwife or fertility specialist.** This sheet makes you an informed partner in that conversation; it is not a prescription.

Evidence table

SUPPLEMENT	WHO / WHY	WHAT THE EVIDENCE SAYS	TYPICAL GUIDANCE (CONFIRM WITH YOUR DOCTOR)
Folic acid	everyone who could become pregnant	Strong: dramatically reduces neural-tube defects; neural tube closes before most pregnancies are confirmed	400 mcg daily from ≥1 month before conception through early pregnancy (ACOG, CDC); 4 mg by prescription if previous NTD-affected pregnancy or other risk factors
Prenatal multivitamin	start before conception	Covers folic acid, iodine, B12, vitamin D, iron	Iodine 150 mcg/day (ACOG); check choline content
Vitamin D	common deficiency (winter, darker skin, indoor life)	Endocrine Society recommends testing and correcting deficiency	Dose based on blood level; take with a meal containing fat
Choline	early brain development	Most prenats fall short	Eggs are the best food source; ask about the prenatal's content
Omega-3 DHA	if not eating oily fish twice a week	Supports fetal brain/eye development	Fish oil or algae; check third-party testing for purity
Iron	only if deficient	Testing (ferritin) first; excess iron is harmful	Away from coffee, tea and calcium; with vitamin C
Men: multivitamin with zinc & selenium	male partner	Cochrane 2019 review: low-certainty evidence that antioxidants improve live birth	Reasonable and inexpensive; food (walnuts, seafood, vegetables) has better evidence

SUPPLEMENT	WHO / WHY	WHAT THE EVIDENCE SAYS	TYPICAL GUIDANCE (CONFIRM WITH YOUR DOCTOR)
CoQ10	egg/sperm quality, often in ART	Improved ovarian response in poor responders in one study (Xu et al., <i>Reproductive Biology & Endocrinology</i>); small, emerging	Specialist decision
Myo-inositol	PCOS	Meta-analysis: improved ovulation and metabolic markers (Unfer et al., <i>Endocrine Connections</i>)	Dose and form matter; specialist decision
Magnesium	cravings, sleep, mild anxiety	Systematic review suggests benefit with limited evidence (Boyle et al., <i>Nutrients</i>)	Greens, nuts, beans first; discuss supplements

Cautions

- **Vitamin A (retinol):** high doses are linked to birth defects; ACOG advises staying well under 10,000 IU/day. No extra vitamin A on top of a prenatal; no cod liver oil unless your doctor says so. Beta-carotene from vegetables is fine.
- **Herbal "fertility blends":** unregulated, may have hormonal effects and drug interactions. A question for your doctor, not a purchase.
- **More is not better.** Nutrients have ceilings.
- **Interactions:** bring every bottle, powder and tea to your appointment.

Quality checklist (before you buy)

- ☐ Third-party tested: USP, NSF or ConsumerLab seal
- ☐ Doses disclosed — no "proprietary blends"
- ☐ Bought from a pharmacy or reputable retailer, not a marketplace seller
- ☐ Contains folic acid (or L-methylfolate if your doctor prefers) at 400 mcg or as prescribed
- ☐ Iodine 150 mcg; vitamin D listed; choline listed
- ☐ Vitamin A as beta-carotene or $\leq$ recommended retinol
- ☐ No ingredients you can't identify

How to take them

Folic acid: any time, consistently · Iron: away from coffee/tea/calcium, with vitamin C · Vitamin D: with a meal containing fat · Prenatal upsets your stomach: try it with dinner · Couples: take yours together at the same meal (shared ritual = kept habit).

Research one supplement (30 XP)

Supplement: _____ Why I'm considering it: _____

Evidence I found (source): _____

Possible downsides / interactions: _____

Question for my doctor: _____

My doctor conversation (bring this sheet)

- "I'm planning to conceive in the next few months. Which supplements do you recommend I start now?"
- "Could we check vitamin D, ferritin/iron, B12 and thyroid?"
- "Is my prenatal's folic acid dose right for my history?"
- "Here is everything I currently take, including teas and powders."
- "For my partner: anything you'd suggest, or tests worth doing?"
- If relevant: "I have PCOS / thyroid disease / a history of miscarriage / I'm vegan — does that change anything?"

What we decided together (40 XP: start only with approval)

SUPPLEMENT	DOSE	WHEN	APPROVED BY	START DATE

Tests requested: _____ Follow-up date: _____

Educational content only, not medical advice. Always consult your doctor or a qualified health professional before changing supplements, diet, exercise or detox practices, especially while trying to conceive.